



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : يسريه عرفات اسماعيل سيد

العنوان : 40 ش عبد المجيد دار السلام - القاهرة

الحي / المدينة : دار السلام

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 05-10-2026

رقم البطاقة : 26912263200087 رقم التليفون : 01007103705

العمر (تاريخ الميلاد) : 26-12-1969 المهنة : حاصل علي بكالوريوس

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: يسريه عرفات اسماعيل سيد National ID: 26912263200087	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
مركز قبطى الشخصية
٢٠١٢



سريه

عرفات اسماعيل سيد

٤٠ ش محمد مجيد

دار السلام - القاهرة

٢٠١٩/١/٢٤

٢٦٩١٢٢٦٢٢٠٠٠٨٧

112279572



٢٠٢٠/٠١ ٢٦٩١٢٢٦٢٢٠٠٠٨٧

حاصله على ليسانس اداب

انثى
مملوكة
متزوجة
مجدى سيد على صلاح

البطاقة سارية حتى ٢٠٢٧/٠١/٢٦





Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m1946

Female : النوع
الهيم : العنوان
03/04/2019 13:55 : تاريخ استلام العينة
09/04/2019 13:59 : تاريخ التقرير

المريض : يسوية عرفت اسماعيل
سن : 49 Y
يغخذ العينة :
الطلب التحليل :
ناتجة : م/الهيم

Primary Sample :

Paraffin Block

Test :

HER2 neu (according to CAP scoring system)

Comment :

Technique :

Sample recieved was treated by : - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :

HER2/neu protein over expression : strong continuous membrane staining of > 10% of tumor cells.

Referral Lab :

Comment :

Conclusion :

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

Pathologist:

د/الان رمزي

Lab Director: د/نور امان

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel (165)7948544 - 7950098 Fax. (202)7948544
Mobile . 01144297292
website: http://www.healthlabs.net
email: pathologychl@yahoo.com
facebook: قسم الباثولوجي - الإدارة المركزية للمعامل

⚠ This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بالعينة لمدة 30 يوم من تاريخ استلامها

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : **بسمريه عرفات اسماعيل**

Age : 49 Y

Referred By Prof. Dr. : **عبد الملاح الشيخ**

Clinical Diagnosis : Left breast mass

Nature of Specimen : Wide local excision and axillary dissection

Receiving Date : 20/04/2019

Analysis ID :  374494

Gender : Female

Delivery Date : 23/04/2019

PATHOLOGY REPORT

Gross :

Two containers were received designated as:

1-Specimen received consists of a piece of yellowish rubbery tissue measured 6.5x5.2x2 cm. No skin ellipse. Cut section revealed firm gritty tumor tissue with ill defined borders measured 1.7x1.6x1.5 cm. The least surgical margins measured 0.2 cm .
Serialing revealed another firm gritty greyish white focus measured 0.4x0.3 cm, located at 1 cm from the main mass .

2-Fibrofatty tissue piece measured 12x12x1.5 cm, dissection revealed multiple lymph nodes with greyish white rubbery cut section, largest measured 2.5 cm in diameter.

Microscopic :

Sections examined from the specimen received revealed breast tissue showing two adjacent foci of tumor tissue showing infiltrating tumour tissue formed of irregular groups of malignant ductal cells showing moderate nuclear pleomorphism, hyperchromasia. There is moderate tubular formation. There is moderate mitotic index. There is peritumoral lymphocytic infiltration about 5%. There are scattered tumor emboli. There is focal tumoral necrosis. There is moderate desmoplasia.

There is intraductal component about 10 % showing solid and cribriform pattern.

The surrounding breast tissue showed intraductal papillae and adenosis.

Surgical margins are free.

Sections examined from the axillary nodes revealed no metastatic deposits in 17 nodes.

The lymph nodes showed intact fibrous capsules, preserved architecture, patent subcapsular sinuses and reactive follicles. The medullary sinuses are patent.

Diagnosis :

Left breast mass, Wide local excision and axillary dissection, **MULTIFOCAL INFILTRATING DUCT CARCINOMA GRADE II, INTRADUCTAL COMPONENT LESS THAN 10%, FREE SURGICAL MARGINS, NO AXILLARY LYMPH NODE METASTASIS, L.N. 0 / 17. PT1c N0**

23/04/2019
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣
فاكس : ٢٥٧٧٣٢٥٦
الدور الرابع و الخامس و السابع
بلوك أ - ميدان رمسيس - ٢٣/٠٤/٢٠١٩

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Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : **بسمية عرفات اسماعيل**

Age : 49 Y

Referred By Prof. Dr. : **عبد الفتاح الشيخ**

Clinical Diagnosis : Left breast mass

Nature of Specimen : Wide local excision and axillary dissection

Receiving Date : 20/04/2019

Analysis ID :  374494

Gender : Female

Delivery Date : 23/04/2019

PATHOLOGY REPORT

Gross :

Two containers were received designated as:

- 1-Specimen received consists of a piece of yellowish rubbery tissue measured 6.5x5.2x2 cm. No skin ellipse. Cut section revealed firm gritty tumor tissue with ill defined borders measured 1.7x1.6x1.5 cm. The least surgical margins measured 0.2 cm .
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Microscopic :

Sections examined from the specimen received revealed breast tissue showing two adjacent foci of tumor tissue showing infiltrating tumour tissue formed of irregular groups of malignant ductal cells showing moderate nuclear pleomorphism, hyperchromasia. There is moderate tubular formation. There is moderate mitotic index. There is peritumoral lymphocytic infiltration about 5%. There are scattered tumor emboli. There is focal tumoral necrosis. There is moderate desmoplasia.

There is intraductal component about 10 % showing solid and cribriform pattern.

The surrounding breast tissue showed intraductal papillae and adenosis.

Surgical margins are free.

Sections examined from the axillary nodes revealed no metastatic deposits in 17 nodes.

The lymph nodes showed intact fibrous capsules, preserved architecture, patent subcapsular sinuses and reactive follicles. The medullary sinuses are patent.

Diagnosis :

Left breast mass, Wide local excision and axillary dissection, MULTIFOCAL INFLTRATING DUCT CARCINOMA GRADE II, INTRADUCTAL COMPONENT LESS THAN 10%, FREE SURGICAL MARGINS, NO AXILLARY LYMPH NODE METASTASIS, L.N. 0 / 17. PT1c N0

23/04/2019
تليفون : ٢٥٧٩٠٩٢٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣
فاكس : ٢٥٧٧٣٢٥٦
بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : يسريه عرفات اسماعيل

Age : 49 Y

Referred By Prof. Dr. : وفاء محمد عاشور

Clinical Diagnosis : Left breast upper inner quadrant mass

Nature of Specimen : US guided tru cut biopsy

Receiving Date : 26/03/2019

Analysis ID :  368048

Gender : Female

Delivery Date : 28/03/2019

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, measured up to 1.5 cm long, totally submitted.

Microscopic :

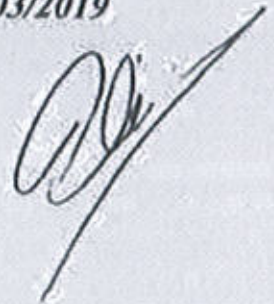
Serial sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia with moderate tubular formation and moderate number of mitotic figures. There is peritumoural lymphocytic infiltrate 10% with marked desmoplasia.

There is minimal intraductal component showing solid pattern.

Diagnosis :

Left breast upper inner quadrant mass, US guided tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II.

28/03/2019



1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٦

Dr.
Elia Anis Ishak

M. Sc, Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc, MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : **بشرية عرفات اسماعيل**

Age : 49 Y

Referred By Prof. Dr. : **ولاء محمد عاشور**

Clinical Diagnosis : Left breast upper inner quadrant mass

Nature of Specimen : Block

Receiving Date : 26/03/2019

Analysis ID :  368049

Gender : Female

Delivery Date : 28/03/2019

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP, DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- Estrogen receptors
- Progesterone receptors.
- HER2 protein.
- Ki 67.

Results :

Sections treated for Estrogen receptors revealed POSITIVE Nuclear staining by chromogen.

Sections treated for Progesterone receptors revealed POSITIVE Nuclear staining by chromogen.

Sections treated for HER2 revealed MODERATE CONTINUOUS CELL MEMBRANE STAINING IN MORE THAN 10% OF TUMOR CELLS

18 % of tumor cells are positive for ki 67.

Conclusion :

Block, TUMOUR CELLS ARE

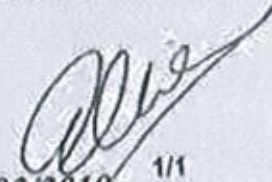
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8 / 8).

POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 6 / 8).

EQUIVOCAL FOR HER 2 PROTEIN OVEREXPRESSION,

SCORE 2+

- CASE FOR CONFIRMATION OF HER 2 GENE AMPLIFICATION BY SISH TECHNIQUE.
THE PROLIFERATION INDEX OF KI 67 IS 18 % OF THE TUMOUR.


28/03/2019

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٦



Patient Name: Yosrea Arrafat Ismail Sayed

Patient Gender: Female

Patient Code: Q36958

DATE: 6/18/2023 9:52:46 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of left breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Increased tracer uptake is noted in LV5 likely degenerative, *for MRI correlation to exclude underlying serious pathology.*
- Degenerative changes of the spines are noted.
- Arthritic changes of knees are noted more prominent on the left side.
- The rest of the surveyed skeleton is unremarkable.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Nahla Dessoki, MD



Patient Name	: الأستاذ / فدييه إبراهيم لصيف عبد السلام	Request No.	: 13000357
Patient No.	: 26608251500645	Request Date	: 16/03/2023
Path No.	: 809/2023	Verification Date	: 12/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left breast mass lesion

Nature of specimen:
Tru cut biopsy, collected at 15/3/2023 , 10:00 am

Gross examination
Multiple greyish white cores up to 1cm , totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
- Histological type : Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism +mitosis) : (3+2+1) Grade II
- IPT :
E-cadherin : Diffuse strong positive staining
P63: Negative staining
Calponin : Negative staining
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
Her-2: No membrane reaction in tumor cells (score 0)
Ki 67: 10 % nuclear reaction in tumor cells

Diagnosis:
Left breast mass lesion, tru cut biopsy:
-Invasive duct carcinoma grade II
-ER : Positive (8/8)

Verified by :
dr.esraa magdy

Printing Date : 16/04/2023 09:15:34

Page 1 of 2

Printed By : marwa mohamed youss



Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name	الأستاذة / فديه ابراهيم لطيف عبد السلام	Request No.	13000357
Patient No.	26608251500645	Request Date	16/03/2023
Path No.	809/2023	Verification Date	12/04/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	56 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- PR : Positive (8/8)
- HER2 : Negative (score0)
- Ki 67: 10 % nuclear reaction in tumor cells

Prof. Gamal Abdelatty
Dr. Esraa Magdy
Dr. Israa-Elgendy

Verified by
dr.esraa magdy

Printing Date : 16/04/2023 09:15:34

Page 2 of 2

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Presidential Program for Women's health

Name: Fadia Ibrahim Lateef AbdelSalam

Date: 15/3/2023

High Resolution Breast Ultrasound Examination

And Revision of poor quality Bilateral Digital mammography done outside our institute.

Clinical SUMMARY : Left breast palpable lump

Detailed Findings:

Left breast

- Dense fibroglandular densities (ACR c).
- UOQ partially irregular partially indistinct mass of equal density.
- Inner central well circumscribed lesion of equal density with internal benign looking pop-corn calcification.
- Ultrasound revealed UOQ hypoechoic solid mass with angulated margin , measures about 28x18 mm .
- 11/12 o'clock oval shaped hypoechoic lesion with internal cystic changes and calcifications, measures 18x8 mm.
- Preserved axillary LNs.

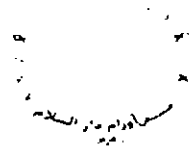
Right breast

- Dense fibroglandular densities (ACR c).
- No masses, architectural distortion, or suspicious group of microcalcifications.
- Ultrasound revealed No solid or cystic mass lesions.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Conclusion:

- **Right breast: (BIRADS 1). normal study .**
- **Left breast: (BIRADS 4b) UOQ palpable solid lesion with angulated margin for biopsy.**
(N:B: biopsy was done at same sitting)

Dr. Marwa M.Hassan



Patient: Name: FDI AH Ibrahim Lotayef Abd Al Salam	Date : 19/04/2023
Exam C.T CHEST WITH CONTRAST	Age :
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Left breast UOQ irregular speculated mass lesion measuring 2.4 x 3 cm.
- No sizable pulmonary nodules, masses or consolidations.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Right lumbar retroperitoneal soft tissue nodule/node measuring about 1.3 x 1.7 cm.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Left adrenal well defined lesion of fat density measuring about 2.4 x 2.7 cm (myelolipoma).
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

Patient Name: FDI AH Ibrahim Lotayef Abd Al Salam	Date : 19/04/2023
Exam : C.T CHEST WITH CONTRAST	Age :
Medical ID:	

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebra' structural collapse or extra-osseous soft tissue component.

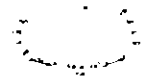
OPINION:

- ***Left breast malignant looking mass lesion as described.***
- ***No definite pulmonary hepatic or osseous metastatic deposits.***

 **Right lumbar retroperitoneal soft tissue nodule/node for follow up.**

Dr. Fifi Samy Mahmoud, MSc.

Dr. ~~Amr Farouk~~, M.D.



Full Name : الأستاذ / فديه ابراهيم لطيف عبد السلام	Request No. : 13004532
Phone No. : 26608251500645	Request Date : 15/05/2023
Path : 1452/23	Verification Date : 01/06/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data : Left breast cancer

Nature of the specimen: Left BCS & Left SLN biopsy

Gross Examination:

- Procedure: Left BCS specimen measured 12x7x4 cm ,
- Tumor extent : A well defined greyish white firm mass measured 2.5x1.3x1.5 cm .
- Distance from margins: 1cm away from inferior margin , 3 cm away from deep margin, 4.5 cm from upper, 1.3 cm from medial, 4.5 cm from lateral, 1.5 cm from superficial margins .
- Lymph nodes number and size: A fibrofatty tissue piece measured 8x6x1 cm enclosing 3 SLNs up to 1.5x1 cm .
- Frozen section: Free margins & Free SLNs (0/3), however in parafin sections SLNB (1/3) without capsular extension

Microscopic examination:

- Histological type: Invasive solid papillary carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Angioinvasion: No
- Ductal carcinoma in situ: present
- Lymph node involvement: SLNs (1/3) showing microscopic metastasis (2 mm) without capsular invasion
- Resection Margins: Free margins

Diagnosis:

Left BCS & Left SLN biopsy:

Invasive solid papillary carcinoma Grade II

- Free LNs (1/3) showing microscopic metastasis (2mm) without capsular invasion
- Free margins
- Stage : pT2 , pN1

Verified by :

Dr.Heba Hasan

Printing Date : 06/06/2023 09:24:18

Page 1 of 2

Printing By : .marwa mohamed youss



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الاستشارة
طبية
مصر

MDT FORM

30/4/2023 G ٢٥

Name	فادي ابراهيم طه	MRN	1234
Age	٥١	ID	٢٦٦٠٨٢٠١٠١٤٥
Menopausal status		Phone	٠١٠٠٣٨٦٠٠٢٨
NRS SCORE		Marital status	
BMI		Family history	no
CO morbidity	DM, HTN	REASON OF MDT	

Clinical Data

BX: 1+ IDC grade II	R	L	
ER: 8/8	PR: 8/8	HER2: -ve	Ki67: 10%
Sonogrammmography:	R L		
1+ breast (BIRAD 4b) with palpable solid lesion	△ () △ ()		
Metastasis → normal study	R CTNM 20		
CTs → free	skin		
Retrospective N. → 1.3x1.2			

Recommendations:

6/5/2023 → For CESM (dense breast) 4 Re MDT

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Mohamed El-Farouk	Dr. El-Farouk	Dr. El-Farouk			



Name: Fadia Ebraheim Lateif

Date: 08/05/2023

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A case of left IDC II

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: minimal background enhancement

ACR: C (heterogeneously dense)

LEFT BREAST:

- **Low energy (Non contrast) images:**
 1. UOQ focal asymmetrical density with distortion
 2. UIQ/ central lobulated isodense lesion with popcorn calcifications seen within
- **Recombined (Post contrast) images:**
- UOQ posterior one third moderate heterogeneously enhancing mass lesion seen inseparable from clumped nodular segmental non mass enhancement extend from the mass to about 3cm from the nipple with overall coverage area of the mass & non mass enhancement 7.5x3.5x1.7cm
- Normal skin / nipple / areolar complex.
- **Complementary US:**
 1. UOQ 12-2 o'clock lobulated mass lesion with adjacent infiltrative parenchyma of altered echogenicity & shadowing & dilated ducts & simple & complicated cysts.
 2. 11 o'clock zone B lobulated hypoechoic mass lesion with popcorn calcifications.
 3. Indeterminate level I 3 discrete axillary lymph nodes, the largest shows focal cortical thickness up to 3.5mm, however it is kidney shaped with preserved hilum .

RIGHT BREAST:

- **Low energy (Non contrast) Images:** UOQ middle one third lobulated isodense mass lesion with popcorn calcifications seen within
- No spiculated masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) Images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
- UOQ 9 o'clock zone B lobulated isoechoic mass lesion with popcorn calcifications

IMPRESSION:

LEFT BREAST:

1. UOQ 12-2 o'clock pathological mass & non mass enhancement (BIRADS6 = pathologically proved malignancy)
2. UIQ 11 o'clock non enhancing benign lesion involuted fibroadenoma (BIRADS2= benign)
3. **RIGHT BREAST:** UOQ 9 o'clock non enhancing benign lesion involuted fibroadenoma (BIRADS2= benign)

Best regards,

Dr. Marwa Ramzy



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : فتيه ابراهيم لطيف عبدالسلام

Sex:

3- Medical number:

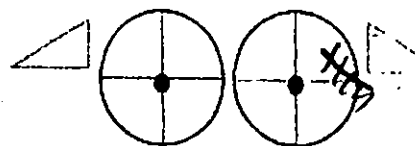
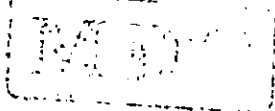
Female

83137136

Old Med :

2-ID number: 26608251500645

Patient Age : 57 Y



Family history

Phone

010064382916

BMI

REASON OF MDT

POST OPERATIVE

marital status

NRS Score

GROUP

B

Date of MDT

2023-06-13

Co morbidity

DM
HTN

Menopausal
status

POSTMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :

BX:
IDC

ER:
8/8

PR:
8/8

Her 2 Neu :
0

SISH :

KI67 :

LT GII 4/23

T
2(2.5CM)

N
1/3

M
0

Sonomammography

PATIENT HISTORY

PATHOLOGY REPORT 1/6/2023-->LT BCS&SLN BIOPSY:

INVASIVE SOLID PAPILLARY CARCINOMA GII,,FREE LN 1/3,,FREE MARGINS,,PT2 PN1

MDT OUTCOME & PLAN

1- ADJUVANT TAXOTERE ENDOXAN X4

2-PORT

2-ADJUVANT AIX5 YEARS

SIGNATURE

MEDICAL ONCOLOGY

د/ علاء حبيب

RADIOLOGY

د/ رانيا شفيق

SURGERY

PATHOLOGY

Psychiatric

Nutrition



Presidential Program for Women's health

Name: Fadia Ibrahim Lateef AbdelSalam

Date: 15/3/2023

High Resolution Breast Ultrasound Examination

And Revision of poor quality Bilateral Digital mammography done outside our institute.

Clinical SUMMARY : Left breast palpable lump

Detailed Findings:

Left breast

- Dense fibroglandular densities (ACR c).
- UOQ partially irregular partially indistinct mass of equal density.
- Inner central well circumscribed lesion of equal density with internal benign looking pop-corn calcification.
- Ultrasound revealed UOQ hypoechoic solid mass with angulated margin , measures about 28x18 mm .
- 11/12 o'clock oval shaped hypoechoic lesion with internal cystic changes and calcifications, measures 18x8 mm.
- Preserved axillary LNs.

Right breast

- Dense fibroglandular densities (ACR c).
- No masses, architectural distortion, or suspicious group of microcalcifications.
- Ultrasound revealed No solid or cystic mass lesions.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Conclusion:

- **Right breast: (BIRADS 1). normal study .**
- **Left breast: (BIRADS 4b) UOQ palpable solid lesion with angulated margin for biopsy.**
(N:B: biopsy was done at same sitting)

Dr. Marwa M.Hassan



Patient Name: FDI AH Ibrahim Lotayef Abd Al Salam	Date : 19/04/2023
Exam : C.T CHEST WITH CONTRAST	Age :
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

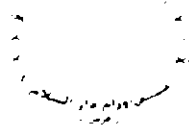
1. Chest:

- Left breast UOQ irregular speculated mass lesion measuring 2.4 x 3 cm.
- No sizable pulmonary nodules, masses or consolidations.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Right lumbar retroperitoneal soft tissue nodule/node measuring about 1.3 x 1.7 cm.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Left adrenal well defined lesion of fat density measuring about 2.4 x 2.7 cm (myelolipoma).
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Patient Name: FDIAH Ibrahim Lotayef Abd Al Salam	Date : 19/04/2023
Exam : C.T CHEST WITH CONTRAST	Age :
Medical ID:	

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast malignant looking mass lesion as described.***
- ***No definite pulmonary hepatic or osseous metastatic deposits.***
- ***Right lumbar retroperitoneal soft tissue nodule/node for follow up.***

Dr. Fifi Samy Mahmoud, MSc.

Dr. Amr Farouk, M.D.